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Dialogue analysis of a filmed Gestalt therapy session: an introduction to a method

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Abstract: Psychotherapy process research takes us beyond answering the question, 'does psychotherapy work?' This area of research aims to study the hows and whats that happen within the psychotherapeutic session that potentiate change. The psychotherapeutic dialogue is an important source of data for psychotherapy process research. Micro-analyses of dialogical turns within the therapeutic session support the understanding of the therapeutic method. This paper introduces the Helbig Method of Dialogue Analysis. This method is founded upon four pillars: 1) that dialogue is implicit action between persons that is supported by explicit verbally uttered content; 2) that the individual's mode of interaction within the dialogical dyad reflects the person's relationship patterns; 3) that dialogue is an intersubjective process that leads to the development of new intersubjective configurations; and 4) that the observer-researcher's phenomenological involvement plays a part in the analytical process. In this study, Bob Resnick's video-recorded Gestalt therapy session entitled 'A Rose on the Grave of my Family' was selected. The transcription of the session was coded using the instrument, the Core Conflictual Relationship Theme – Leipzig/Ulm. Results obtained from this study are quantified graphical representations of the developing relationship between therapist and client. Simple to operate, scalable and practical, this method is designed for use by therapists and researchers who are interested in tracking, comparing and/or contrasting the developing psychotherapeutic alliance in a single or in multiple psychotherapy sessions.

Keywords: psychotherapy process research, dialogue analysis, psychotherapeutic alliance, Gestalt therapy.

Introduction

As Gestalt therapists, the 'dance' between therapist and client is central to us in supporting therapeutic change. Four decades of psychotherapy research has already validated this very idea (Hovarth, 2006; Lambert, 2013; Wampold, 2001), that the psychotherapeutic alliance (PA) is an essential variable in psychotherapy research. The developing PA is largely intangible. It is sensed rather than seen or heard. How can we then measure, compare and contrast such an important variable like the developing PA from a sample of psychotherapeutic dialogue? This is the challenge posed to myself as I developed this method of analysing dialogue for my Master's thesis. I was advised by my supervisors to name this method the Helbig method of dialogue analysis (HELDA).

This method is founded on the following premise: 1) that dialogue is implicit action between persons that is supported by explicit verbally uttered content; 2) that how the client interacts with the therapist in the session is a reflection of his/her general relationship pattern; 3) that dialogue is an intersubjective process from which evolve new intersubjective configurations; and 4) that the person of observer–researcher is involved in the analytical process.

Empirical science seeks the quantifying of data. In order to measure the PA, I needed to convert transcript of the dialogue – which is 'the data' – into quantifiable pieces of code. The HELDA method involves deconstructing a sample of therapeutic dialogue into discrete meaningful units and then attaching codes to these units. The research instrument by Albani et al. (2008) called the Core Conflictual Relationship Theme Leipzig/Ulm (CCRT-LU) was chosen to code the sample. Through coding the data, we created visual representations of the developing PA with time.

In the study featured in this article, Bob Resnick's video entitled 'A Rose on the Grave of my Family' (Resnick, 2016), which is publicly available, is studied

using this method. It being a representative piece of Gestalt therapy work, readers of this article may be able to connect with what is felt from observing the evolving PA with the graphical results generated by the HELDA method.

As I write this introduction, I can hear the voices of some readers asking me, 'why do this?' I have an inherent need to understand what I see, especially if what I see inspires me. The video presented in this study is an example of such material. Transcribing and coding the dialogue is a process of making contact with the work. The results obtained allow me to communicate what I have assimilated back to the environment.

What are we seeing?

Observing a video recording of a therapeutic session such as 'A Rose on the Grave of my Family', we attain two sets of information: the recorded dialogue between therapist and client and how we feel while viewing the session. I have viewed this film on several occasions with different groups of fellow therapists and psychotherapy interns. In each viewing, viewers almost unanimously agreed that the therapist, Resnick, through his interventions, had managed to build a working alliance with the initially dismissive client. Usually viewings of such films inspire lively discussions as the observers share their thoughts and feelings about what they had seen. Qualitatively analysing the dialogue through reflections from such observers provides us with a depth of information. In research, however, there is often a need to compare and contrast different sessions, therapists or observer viewpoints. Such data requires quantification. Researchers need to be able to illustrate what they have seen and sensed into graphs.

The endeavour here is therefore to quantify, within reasonable means, the evolving psychotherapeutic alliance observed in Resnick's video.

The psychotherapeutic dialogue

Why study dialogue? Decades of psychotherapy research have shown that effective outcomes in therapy are related to what is known as 'common factors' (Budge and Wampold, 2015; Lambert, 2013). The personality of the therapist and his/her ability to interact with the client's personality is an implicit common factor that determines outcome in all modalities of psychotherapy. The psychotherapist's only tool is ultimately his/her own person. The therapist uses this tool to be in dialogue with the client. As an observer–researcher, if we were to study a piece of therapeutic interaction non-intrusively, the dialogue is all we have as a sample. If we wish to understand what we have just witnessed in the filmed therapist-client interaction, the dialogue holds the data.

What, then, does this data contain? Therapeutically effective dialogue is inclusive of the self and the other. Dialogue that has transforming power, says Buber (1936/1970), is free of 'the magnitude of contents' (p. 36). It belongs to the present. Its speakers are met through grace, and it is 'an act of being' (p. 11). To identify transformative material from a sample session, the psychotherapeutic dialogue needs to be, to a certain degree, deconstructed.

Elements of psychotherapeutic dialogue

The psychotherapeutic dialogue is different from other ordinary dialogue. In most ordinary conversations, verbal and non-verbal language is used to convey the speakers' communicative intentions. In psychotherapy talk, there is an almost universal intent of the psychotherapist to examine the talk beyond its obviously intended meaning (Peräkylä, 2013).

In research, focus is put primarily on the client's narratives. Clients' explicitly spoken narratives can be differentiated into two broad categories (Bercelli, Rossano and Viaro, 2008): 1) The structural-textural narrative which is made up of the client's account of past experiences, usually presented as a temporal sequence of events and actions; and 2) The interactional narrative which is the client's account of his/her interaction with the self or his/her interactions with other people. How these narratives are told is co-created. How the client narrates these stories is not only dependent on the speaker, it depends also on the therapist who is listening (Sacks, 1992). This other dimension in psychotherapy talk is what we term the non-narrative psychotherapy talk.

Non-narrative psychotherapy talk is the implicit interactional exchange in the here-and-now between therapist and client as the conversation takes place. In this method, the explicit contents in the client's narrative and the implicit non-narrative interaction between client and therapist in the talk are differentiated. To identify the former, we ask, 'what is the client saying to the therapist?', while to identify the latter, we ask, 'what is the client doing with the therapist?' In other words, when a client tells his therapist a story, the story has as much to do with him, the speaker, as it has to do with the listener, the therapist.

What I have proposed with this method is that one thinks of dialogue as implicit interaction between speaker and listener that is supported by the speaker's explicitly spoken content. In the process of measuring the evolving psychotherapeutic alliance, it is the implicit aspect of dialogue that is of interest and quantified.

How is the implicit interaction differentiated from the explicit content in a sample of dialogue?

A well-known group of researchers, the Boston Change Process Study Group (BCPSG),[1] have in their literature differentiated the implicit and explicit aspects of dialogue. This is how they explain this differentiation:

(i) The *explicit process*: which is the declarative, verbal content matter of interpretations that alter the conscious understanding of the patient's intra-psychic organisation. It is the work of analysis that aims to bring to consciousness what is repressed.
(ii) The *implicit process*: which is procedural and operates outside both focal attention and conscious verbal experience. This knowledge is represented symbolically in what the group calls 'implicit relational knowing' (BCPSG, 2002).

See also Table 1.

The implicit processes in dialogue can be understood as the level of communication that goes beyond the level of content of the speech, relating to the intersubjective interaction that happens during a dialogue. This is supported by the explicitly spoken content.

Methodology

Transcribing and tabulating

In working out this method, the challenge was first to make the differentiation between these two aspects of the dialogue being studied. Both implicit and explicit processes exist concurrently. Hence, a table in Microsoft Excel was created to do this (see Figure 1).

The sample dialogue from the film is transcribed. Both verbal and non-verbal communication is recorded. Each meaningful unit of utterance or expression is input into a single cell in the table. The time is noted. For this study, the session is divided into 9 segments, T1 to T9. Each time segment lasted about 2.8 minutes.

For this study, I analysed both client and therapist's utterances. Each meaningful unit of communication is tagged first with keywords or a phrase that describes the explicit content in column I. For the observer to decide on what to add to this column, one has to ask the question, 'what is being said?'. For example, in line 3, the client spells out the word 'pass' (P-a-s-s). The explicit content in column I would be recorded as 'spelling pass'. This is the literal meaning of the client's utterance.

To fill in column G, the implicit action, the observer asks the question, 'what is the speaker doing?' The observer senses that the client, in spelling out the word, was being cynical and dismissive. So, in column G, the words 'being dismissive' were recorded in the table.

Coding

The goal of this study is to quantify the implicit action of the dialogue. Now that I have differentiated what is implicit action and what is explicit content in the dialogue, the next steps require the quantification of the implicit action. To do this, the entries in column G, the implicit action, need to be assigned code.

The act of coding simplifies what is otherwise natural human interaction into discrete bits of information. Ultimately, coding converts life into numbers. Client narratives and therapeutic dialogues, as Freud himself put it, can be 'read like short stories', but these 'lack the serious stamp of science' (Freud and Breuer, 1895, p. 160). Coding is thus a kind of necessary evil in psychotherapy research.

Coding requires the use of an instrument. After quite a bit of search, I decided to use the instrument, the Core Conflictual Relationship Theme – Leipzig/Ulm (CCRT-LU) to code the transcript. The CCRT was first introduced by Luborsky (1977) and further developed and validated by the Leipzig/Ulm group led by Albani et al. (2008). The CCRT was created with the intent of measuring recurrent relationship patterns of the speaker reflected in his/her narratives. The CCRT-LU code is used to tag the speaker's consistent relationship pattern or schema.

The CCRT-LU codes consist of adjectives that describe human interactional behaviour. These adjectives are

Table 1. The explicit and implicit aspects of dialogue

The explicit	**The implicit**
Words	Action, enactment
Content	Process
Symbolic representation	Non-symbolic representation
Undoing repression, rendering conscious the unconscious	Change in psychological structures
Mutative information for the patient	Mutative relationship with therapist

	A	B	D	E	F	G	I
1	Line	TIM	Speake	Text	CCRT	IMPLICIT ACTION	EXPLICIT CONTENT
2	1	T1	C	I pass.	M12 keeping c	withdrawing/rejecting	"pass"
3	2	T1	T	I don't understand the word.	K22 dominatin	testing someone	T not understanding "pass"
4	3	T1	C	P-a-s-s.	J22 declining,	being dismissive	spelling "pass"
5	4	T1	T	[I know how to spell it]	L12 being gloa	being cynical	T knowing how to spell "pass"
6	5	T1	A	[((raucous laughter for 15s))	L12 being gloa	laughing at	laughing at T-C
7	6	T1	T	[((chuckles along then stops smiles))	D27 having sel	getting serious	stops laughing
8	7	T1	C	((stops smile turns head to T))	D27 having sel	getting serious	stops laughing
9	8	T1	C	What you don't understand?	K22 dominatin	testing someone	what T does not understand
10	9	T1	T	Ah(0.3) I don't understand	D14 being calr	being calm / patient	T does not understand
11	10	T1	T	((leans forward))	C11 being clos	approaching	non-verbal (lean forward)
12	11	T1	T	and I'm being serious now	D17 being sens	being sensible	T being serious
13	12	T1	T	I don't understand uh (.) what gets in the way for you to say	B11 explaining	explaining thoughts	T does not understand
14	13	T1	T	where you are at the moment. uh (.)	B12 standing b	stimulating dialogue	C's not sharing with group
15	14	T1	T	That's what I don't understand,	B11 explaining	explaining thoughts	T does not understand
16	15	T1	T	and I would like to.	B12 standing b	encouraging dialogue	T's interest to understand
17	16	T1	C	((hand gesture))	E21 giving up,	resigning	non-verbal (hand gesture)
18	17	T1	C	I don't know	G13 not being	being uncertain	C don't know
19	18	T1	C	I go in and out	G13 not being	being uncertain	C going in and out
20	19	T1	C	and so many stories and things like that	G21 being wea	being overwhelmed	many other's stories
21	20	T1	C	so I just -- I [just want to pass.]	M12 keeping c	feeling need to retreat	C wanting to withdraw
22	21	T1	C	[((schrugs))]	E21 giving up,	resignation / giving up	non-verbal (shrug)
23	22	T1	C	that's it. w..((hands fall on to lap with a clap))	G21 being wea	showing resignation	that's it
24	23	T1	T	((nodds))	C11 being clos	accepting	non-verbal (nodd)
25	24	T1	T	So you have some objection to.	B12 standing b	stimulating dialogue	C's objection to share

Result | Res-Overall | Res-Client | Res-ClientTherapist | Res-Therapist | Res-ClientSelf | Res-# | Res-LOV | Res-nonLOV | TRANs | DATA-C

Figure 1. Tabulated in Microsoft Excel, the transcript of the sample of dialogue and other variables. Column A lists the line number. Column B lists the time interval. Column C records who speaks (C is client, T the therapist, and A is the group in the background). The dialogue is transcribed in column E. Each meaningful unit of utterance is tabulated into a single cell. The implicit action and explicit content analysed from the transcript are recorded as a keyword or phrase in columns G and I respectively. Subsequently, the contents of column G are coded using the instrument CCRT-LU.

grouped into a hierarchy of categories. The adjective that corresponds best to the implicit action keyword or phrase in column G is chosen for column F shown in Figure 1. In doing so, the implicit action for that piece of transcript is given a code. Taking line 3 again as an example, the client spells out the word 'pass'. The observer senses that she was being cynical and dismissive with this utterance. The observer thinks that a likely code for this implicit action is J22 (declining, excluding, criticising, admonishing, rejecting, judging, rebuke), and enters 'J22' into column F.

This process is repeated throughout the transcript on both the client's and the therapist's words and gestures. The underlying premise falls somewhat in line with the Gestalt therapy concept of co-creation: that the implicit interaction of the client and therapist in the therapeutic session reflects the relationship theme of both persons within the dyad.

As can be seen with the example of line 3, the code assigned to the client's words is J22. This code is assigned mainly because its adjectives, 'declining', 'excluding' and 'rejecting', best describe what the observer has sensed about the client's action. J22 belongs at the bottom of the code hierarchy in CCRT-LU. The codes are further categorised. The topmost categories are what we eventually use for creating the graphs.

The topmost categories are named: they are 'loving', 'strong', 'weak', 'unpleasant', 'fighting', and 'leaving'.[2]

These categories describe how the speaker, through his or her talk, moves away from or moves towards contact with the listener (see Figure 3).

Using Excel, all relevant data was input into tables. Pivot tables, which are a feature of the software, can be generated as shown in Figure 2. These pivot tables are very useful because with these we can further define our study by selecting and deselecting information using the filters. Also, researchers can use the tables to generate graphs and charts according to particular aspects of the dialogue they wish to investigate.

Results

How do the charts reflect the evolving therapeutic alliance of the filmed session? Using different filters on the pivot tables, one is able to tease out attributes of the session that may be interesting. In this article, attributes of the 28-minute recorded interaction between client and therapist from the Resnick video are: 1) How the client's relation with the therapist during the session is represented; 2) How the therapist's relation to the client during the session is represented; 3) How the therapeutic alliance is represented; and 4) How the results align with Gestalt therapy theory.

Speaker	(All)
CONTENT	(All)
RELATION	(All)

Overall Relationship Theme of the Session

CCRT	Time Interval									
Code	T1	T2	T3	T4	T5	T6	T7	T8	T9	Grand Total
⊞LOVING	30,99%	50,00%	37,35%	54,55%	57,89%	46,03%	64,10%	73,61%	88,51%	56,65%
⊞STRONG	28,17%	25,76%	38,55%	18,18%	15,79%	3,17%	16,67%	9,72%	10,34%	18,83%
⊞WEAK	21,13%	7,58%	20,48%	25,45%	24,56%	47,62%	19,23%	16,67%	1,15%	19,46%
⊞UNPLEASANT	9,86%	9,09%	0,00%	0,00%	0,00%	1,59%	0,00%	0,00%	0,00%	2,22%
⊞FIGHTING	7,04%	0,00%	2,41%	1,82%	0,00%	0,00%	0,00%	0,00%	0,00%	1,27%
⊞LEAVING	2,82%	7,58%	1,20%	0,00%	1,75%	1,59%	0,00%	0,00%	0,00%	1,58%
Grand Total	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**	**100,00%**

Figure 2. This is a pivot table created in Excel from the coded transcript. The topmost CCRT-LU categories are considered here. This figure shows the percentage of the total occurrence of each category during each time interval.

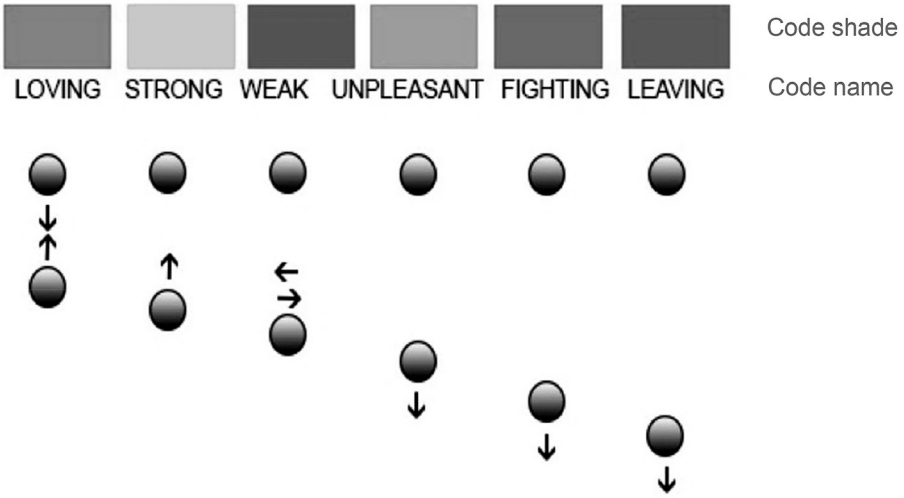

Figure 3. Topmost codes of the CCRT-LU are assigned different shades. The bottom circles represent the speaker. Top circles represent the corresponding listener. The arrows represent the direction towards or away from contact that the speaker makes with the listener in each category.

Client's relation with the therapist intra-session

How the client's CCRT evolved in relation to the therapist is seen in Figure 4. In this chart, filters are set to determine how the client's relational theme changes during the session. The lines now tell the story.

At the beginning of the film, the client seemed to be dismissive. She seemed to be avoiding contact with the therapist. At time interval 2, we see a bump in the 'leaving' and 'unpleasant' lines. The 'leaving' line describes actions withdrawing from contact, while the 'unpleasant' line describes the client's active pushing away contact. There is a sense here of a ruptured alliance.

Looking at the 'weak' line, we have a sense of how the client had set herself in a subordinate position relative to the therapist. 'Weak' codes of the CCRT-LU describe actions of being resigned, anxious, subjected, etc. This is despite the observation that the client had held a tough stance against the therapist's approach at this time. The client's display of weakness trended downwards as the session approached time interval 7 and remained at a low level until the end of the session. We can see from the graph that the client's 'weak' codes were replaced at time interval 7 with the 'strong'. 'Strong' in this context describes the client's ability to introspect and cope. Had the client found some self-support here at time interval 7? Perhaps. From time interval 7 on, we see the 'loving' line emerging skywards. The client is seen to relate to the therapist at the final minutes of the session in a more contactful way. 'Loving' describes actions like showing acceptance, curiosity and gratitude. With more self-support, the client was able to move towards contact.

Therapist's relation with the client intra-session

What does the therapist's chart look like? What can such a chart tell us about the therapist or his/her method in the session? The graph obtained is seen in Figure 5.

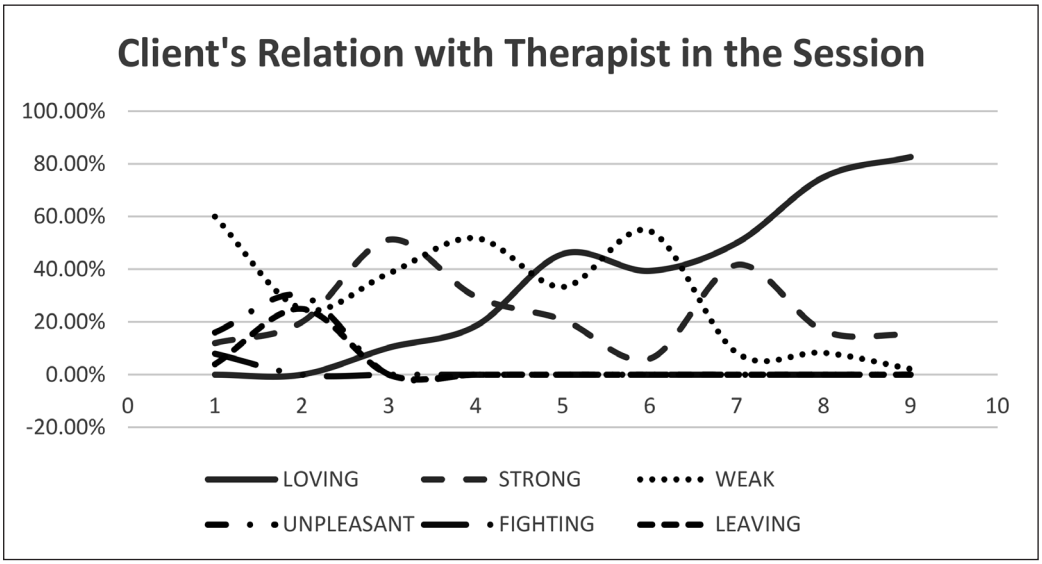

Figure 4. Trend showing the client's relationship with the therapist.

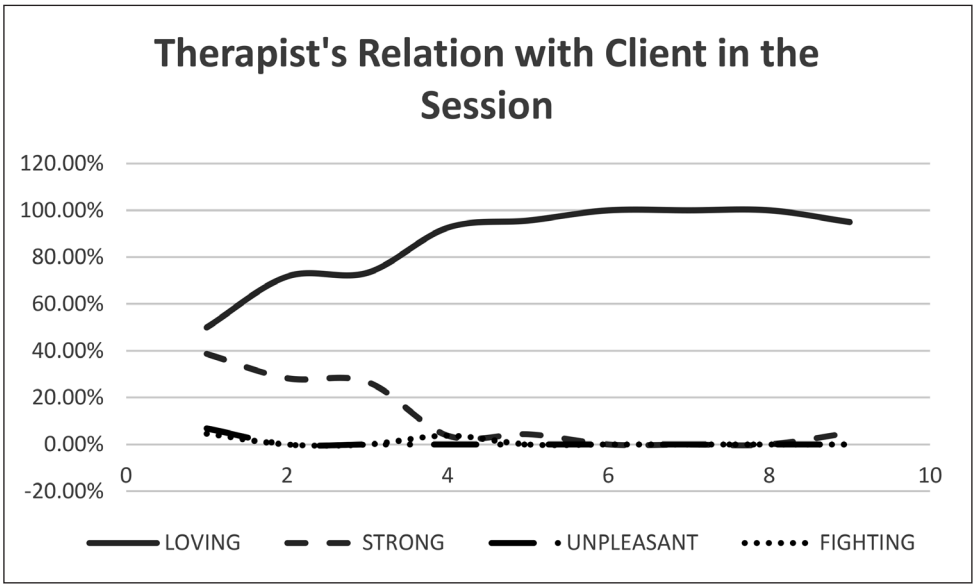

Figure 5. Graph showing the therapist's relation with the client in the session.

With the 'loving' codes dominant throughout, the graph shows the therapist as having maintained a stance of being curious and inviting during the session. From the beginning until time interval 4 of the session, we see the 'strong' line. This line describes how the therapist had taken also a dominant stance at the beginning. In the film, we see the therapist confront the client's withdrawing attitude in their interaction.

Studying the therapist's relationship theme in the session could possibly be useful in understanding the therapist's treatment strategy in a particular session.

How does the therapeutic alliance in this dyad appear graphically?

Filtering out the data to select only the instances of contactful exchange from both therapist and client, we may be able to visualise graphically how the psychotherapeutic alliance developed in the duration of the session recorded. Figure 6 shows a graphical comparison of the number of 'loving' codes of both client and therapist in the 28-minute recording.

'Loving' codes describe contactful interaction of the speaker with the listener. Adjectives describing acceptance of the other, expressing and allowing feelings and curiosity are found in this category of the CCRT-LU. The graph points to a move from rupture in the alliance at the beginning of the session towards repair and contact at the end.

How do the results align with the Gestalt therapy theory of the five phases?

My decision to use this particular video by Bob Resnick to introduce the method was far from a random idea. I was looking for a sample of work that

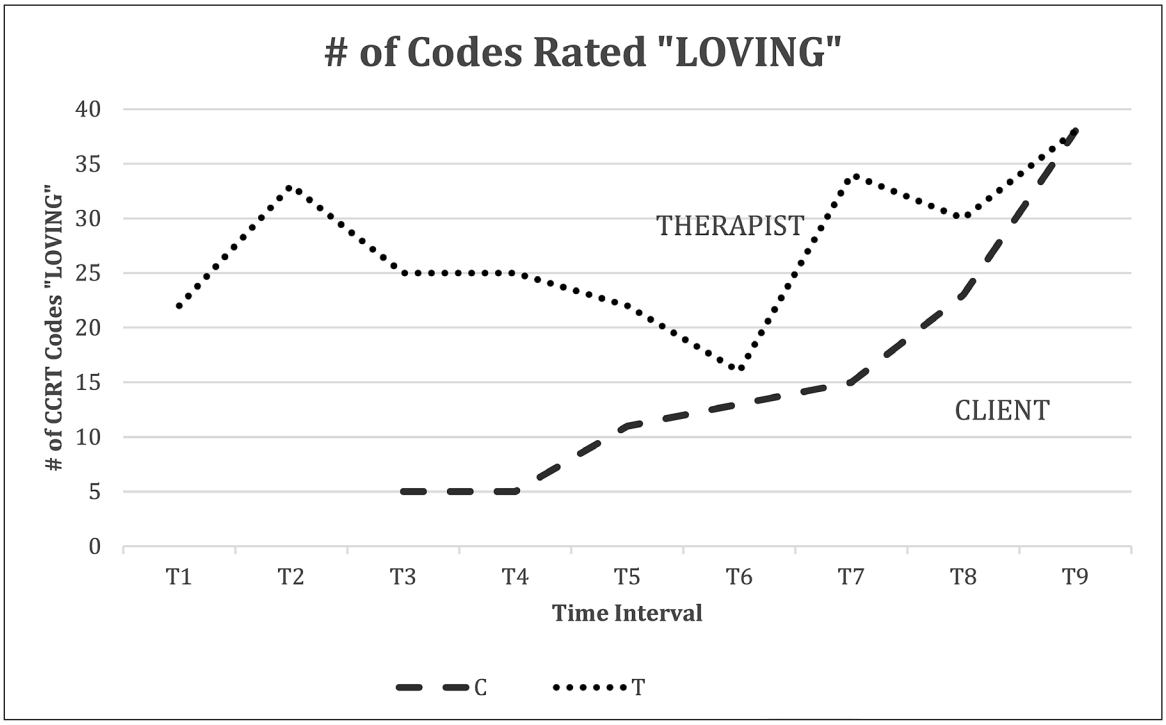

Figure 6. Graph showing the trend in number of 'loving' codes from the therapist's and client's utterances.

is considered by the Gestalt community as being true to the theory of Gestalt therapy. In future research, I would administer the Gestalt Therapy Fidelity Scale by Fogarty et al. (2016) to validate the method in the sample being studied. I was also looking for a piece of work that was publicly available. The video was also only 28 minutes long. This posed a challenge to collect enough data for study, but it also was an effort-saving advantage for me. One thing that stood out for me and the colleagues who viewed the film was how concise the work was. The figure sharpening the move towards contact and the assimilation phases were perceivable. My colleagues and I aligned Resnick's work with the client in this video with the Gestalt therapy five phases.

The five phases of therapeutic change were first described by Perls and Andreas (1969) as the *five layers of neurosis*. Polster and Polster (1978) describe the eight stages of contact (an expanded version), and Votsmeier-Röhr and Wulf (2017) describe the *phases* within the therapeutic process. Here, we consider the completion of the five phases and describe the possible occurrence of them in the context of the sample video as seen in Table 2.

The curiosity here is, how do the graphs align with what is phenomenologically understood and observed from watching the film? The following graphs in Figures 7 and 8 are what we obtained from this exercise.

Figure 7 shows the graph from Figure 6 overlaid with the Gestalt five phases. The point of the impasse at phase 4 shows an interesting dip in the number of codes from the 'loving' category by the therapist. The vacuum phase of the impasse is an emotionally difficult part of the process in therapy. The therapist, in the context of this sample, seemed, from the graph, to have communicated fewer of the 'loving' adjectives to the client. Did the therapist show a less contactful attitude at this crucial time? How did the therapist deal with the impasse? Referring back to the graph in Figure 5, the therapist in this time interval of T5 and T6 communicated predominantly with 'loving' adjectives. The dip in the therapist's line in figure 7 at phase 4 indicates that the therapist remained in contact with the client by saying less, being more silent, and giving more space for the client to speak.

Figure 8 shows the client's change in interaction style with the therapist through the 5 phases.

The graph in Figure 8 demonstrates how the client's interactional attitude towards the therapist changed with time. During the early cliché and differentiation phases, we see how the client interacts with her environment. This is a reflection of the client's relationship pattern pre-therapy: showing less accepting of contact with the others (low value in the 'loving' line), relating to the other in a kind of role play of dominance and sub-dominance ('weak' and 'strong' line alternating), actively defending her space (a trace of the 'fighting' and a bump in the 'unpleasant' lines), and withdrawing (with a bump in the 'leaving' line). Note that her style of relating at the beginning of the session was towards persons whom she had already got to know, not strangers. We may consider that this could be her general pattern of relating to other people. If this were a clinical case study, this would have been investigated.

Table 2. Phases of therapeutic process. Terms of the phases are as per Votsmeier-Röhr and Wulf (2017), and descriptions of these phases (in quotation marks) are from Polster and Polster (1978, p. 196).

Phases	Time Interval	Comment
Phase 1: Cliché 'The emergence of the need, and the attempt to play out the need.'	T1	The client avoids speaking in the group. The therapist confronts her.
Phase 2: Differentiation 'The mobilization of the internal struggle.'	T2, T3	The client decides to come forward to work with the therapist, saying that she was hurt by his confrontation. She says that she is anxious.
Phase 3: Diffusion 'The statement of the theme incorporating the need and the resistance.'	T4	The client accepts the feelings of difficulty in sharing her needs. She is still sceptical, but she stays in conversation with support of the therapist.
Phase 4: Vacuum 'The arrival at the impasse, and the climatic experience.'	T5, T6	The client recounts her childhood experiences. Difficult feelings are expressed.
Phase 5: Integration 'The illumination, and the acknowledgement.'	T7, T8, T9	The client is now able to talk more fluidly. She expresses relief of the insight and warmth of having shared her experiences with the therapist and group.

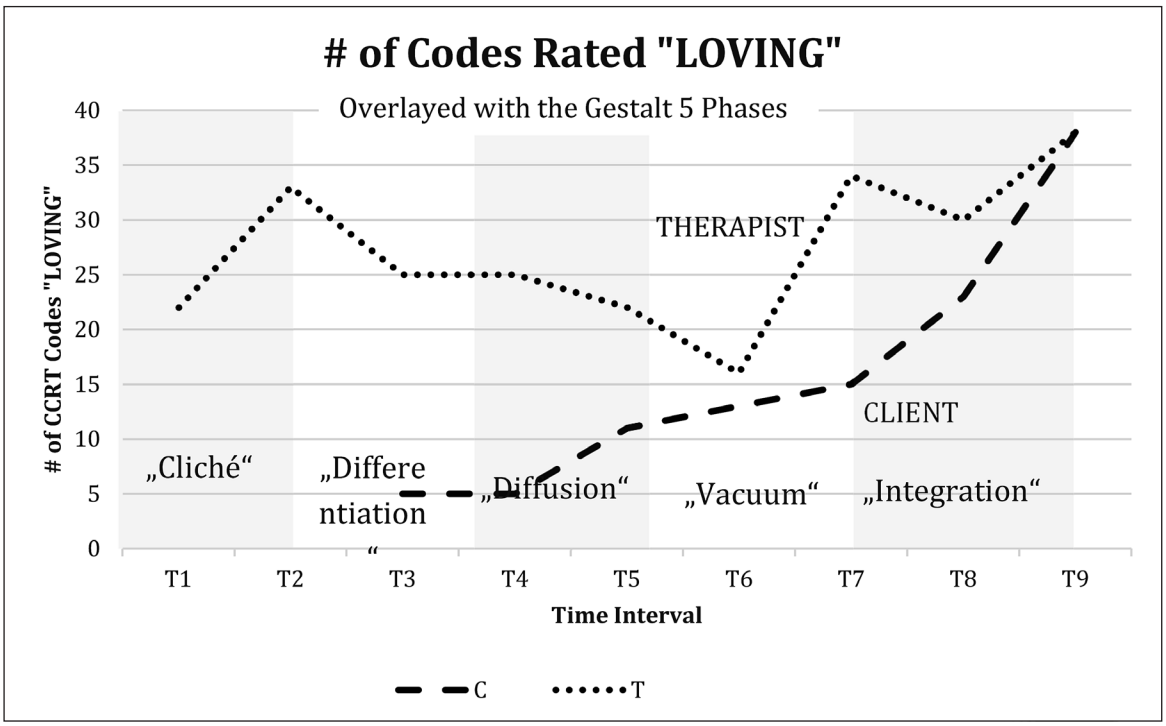

Figure 7. The five phases of Gestalt therapeutic process overlaid onto the graph in Figure 6.

At the diffusion phase, we perceive the client as being more responsive to contact (the 'loving' line ascending). She has also let go of defending and withdrawing (flat 'unpleasant' and 'leaving' lines). She engages the therapist in dialogue, sharing how she was feeling at the present moment, how she felt about being seen, and her scepticism about working with the therapist.

The vacuum phase finds us, the observer watching the film, moved. At this point of the session, the client recounts her childhood and family history. The therapist says little at this point, as seen and explained in Figure 7. The emotion transmitted in the field at this phase of the session felt like grief. This is probably represented by the codes that describe 'weak'. We can

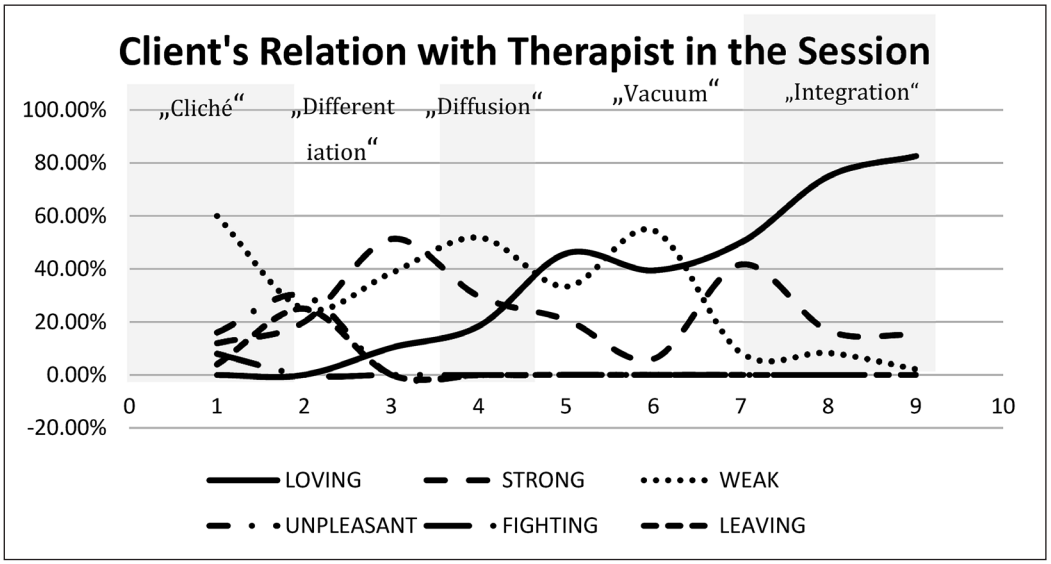

Figure 8. The client's change in interaction with the therapist (as per Figure 4) aligned with the Gestalt five phases.

also see how at this vacuum phase, the client finds the self-support to move towards contact with the therapist. Represented by the 'loving' line, the client interacts with the therapist with more curiosity, acceptance and gratitude.

The final phase finds the client addressing the therapist. She is visibly touched, and thankful. As the session ended, she summarised her experience in metaphor, as if to assure the therapist of her experience. The observer gets a sense of the erstwhile withdrawing client moving in the direction of contact and staying in contact till the end of the integration phase. On the graph, we see the 'loving' line rising.

Conclusion

Having presented this method at the 4th Gestalt Research Conference in Santiago in May 2019, I realised that I am not the only Gestalt therapist–researcher who has an interest in putting the implicit into graphs. Perhaps there lies amongst us a need to support the idea that we can explain the relational part of our therapeutic work quantitatively. This is, in essence, our attempt to speak the language of the larger scientific community.

When I decided to work on this method, the premise was to create something that is non-invasive, scalable, quick to administer, practical and can be used by clinicians, researchers or by anyone who needs to track the evolving psychotherapeutic alliance. It was only as I was coding this first video that I realised how important it was to differentiate the implicit interaction from the explicit content in the dialogue. I also realised that finding the codes for the implicit content required, to a large extent, my personal feelings, beliefs and prejudices at the moment of coding. This is the kind of scientific paradigm that relates to research in the humanities which involves a certain degree of constructivism–interpretivism (Gelo, 2012).

I have, in the meantime, studied transcripts of my own clinical work, many of which were analyses of a series of sessions. This helped me immensely in my work as a clinician, because I could now visualise my own otherwise ego-syntonic patterns of relating to the client.

It is beyond the scope of this article, which is a single-session study, but when analysing a series of sessions by following the same client-therapist dyad with time, I was able to expand this method further. The added data allowed for tracking the development of the client's relationships with significant others in his/her life, or more importantly, with the self. Could this imply a means of visualising outcome of therapy? If so, is this also a way of redefining psychopathology and the therapeutic process? Imagine, for example, a client who self-harms who benefits eventually from therapy. Imagine how his/her graphs pertaining to his/her self-self relationship pattern would look like with time, if the outcome is positive.

Using this tool to visualise the evolving relationship patterns in the psychotherapeutic relationship leaves us ultimately with graphical images. In an Excel program, these images can be shown in colour. This is a way of viewing the therapeutic contact through an aesthetic lens. Lines moving across a horizontal plane can tell us much of the journey of therapist and client which can be grasped at a glance. The observer-researcher, impacted by the sessions, becomes a kind of portrait painter of the therapeutic process. I can imagine that if we were to add a dimension or more to this method, we may even come close to painting impressions of the field itself.

My journey with research is still in progress. It is a slow process as I am an independent researcher. I had not envisaged the positive interest for this work that I received before I set foot in Santiago last May for the 4th Gestalt Research Conference. Aware of the challenges faced by the Gestalt community today, I believe that simply publishing individual projects such as this one could inspire like-minded individuals; people like myself who would have otherwise just sat on their ideas, not realising the impact it can have on the field.

For this, I thank the Gestalt community and the BGJ for offering me a chance to feature HELDA. I look forward to sharing more information about the method, and welcome anyone to contact me directly for information or the related software.

Notes

1. The BCPSG is a combined effort of (alphabetically): Bruschweiler-Stern, N., Harrison, A.M., Lyons-Ruth, K., Morgan, A.C., Nahum, J.P., Sander, L.W., Stern, D. and Tronick, E.Z.
2. I do sometimes find the names given to these top-level codes (provided ready-made by the developers of the CCRT-LU) somewhat misrepresentative of the function intended of these codes. I hope the Gestalt therapy-orientated reader considers this: that the codes describe a movement towards or away from the contact with the listener or the environment. 'Loving' could be considered to be moving towards contact, 'strong' as holding back from contact, 'weak' as avoiding contact, 'unpleasant' as dismissing contact, 'fighting' as pushing away contact, and 'leaving' as a move towards isolation. This will be changed with further development of the HELDA method.

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